SACTCHECK · NCCP 00388

My complete treatment handbook

Oral etoposide · Days 1–3 twice daily

All information can be read on paper. Keep this with your treatment record.

> Use the urgent instructions and contact numbers in the following companion pages. Example telephone numbers must be replaced by your team before patient use. Your individual treatment plan takes precedence.

Contents

1. Your three-page treatment companion — page 2
2. Full treatment reference: medicines, schedule, tests, symptoms and follow-up — page 5
3. Detailed treatment and monitoring plan — page 20
4. Supportive medicines and care — page 23
5. Visits, access and practical arrangements — page 24
6. Your treatment passport and symptom record — page 25
7. Wallet card and phone instructions — page 30

SACTCheck v0.84.31 · Clinical, pharmacy and patient review pending. Formal consent is supplied separately.

Oral etoposide · Days 1–3 twice daily

This treatment aims to slow or shrink extensive small-cell lung cancer when intravenous or combination chemotherapy is unsuitable. Benefit varies; your team explains the aim for you.

> This guide covers only Days 1–3 twice daily. Do not combine it with another etoposide schedule.

My overall treatment

Take your prescribed etoposide capsules twice daily on Days 1–3. Days 4–21 have no planned etoposide. The source cycle is 21 days; continue only while helping and tolerable, after team review.

My capsule schedule
Days 1–3 twice daily; 21-day cycle. Follow the exact prescription.

Review
Your team confirms each course and next start date; no fixed total cycle count.

A typical 21-day cycle

Day 1–3	Day 4–21
Treatment days	Between treatments

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

My tests and checks

When	What is checked	Timing / reason
Before treatment	**Blood count, kidney and liver tests** Required by regimen	Before the first course
During treatment	**Blood count, kidney and liver tests** Required by regimen	Regularly; the team specifies dates for your selected capsule schedule
During / after treatment	**Scans, review and any tumour markers** Team-agreed plan	Consultant-agreed dates; no fixed scan or tumour-marker schedule in this regimen

Between treatments

- Blood counts can fall after dosing ends; fever, bleeding or feeling suddenly unwell needs an immediate call.
- Recovery is checked before another cycle; no calendar day guarantees that your counts or symptoms have recovered.

Confirm capsule strength, number, dosing days, storage and contact numbers with the pharmacist. Have the planned blood tests and tell the team about symptoms and all medicines. Keep the written prescription and symptom record; start the next cycle only after team confirmation.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321
EXAMPLE NUMBERS ONLY — your team must replace these before patient use.

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hair / energy

Etoposide

Hair loss and tiredness may occur; report marked weakness or dizziness.

2 Mouth

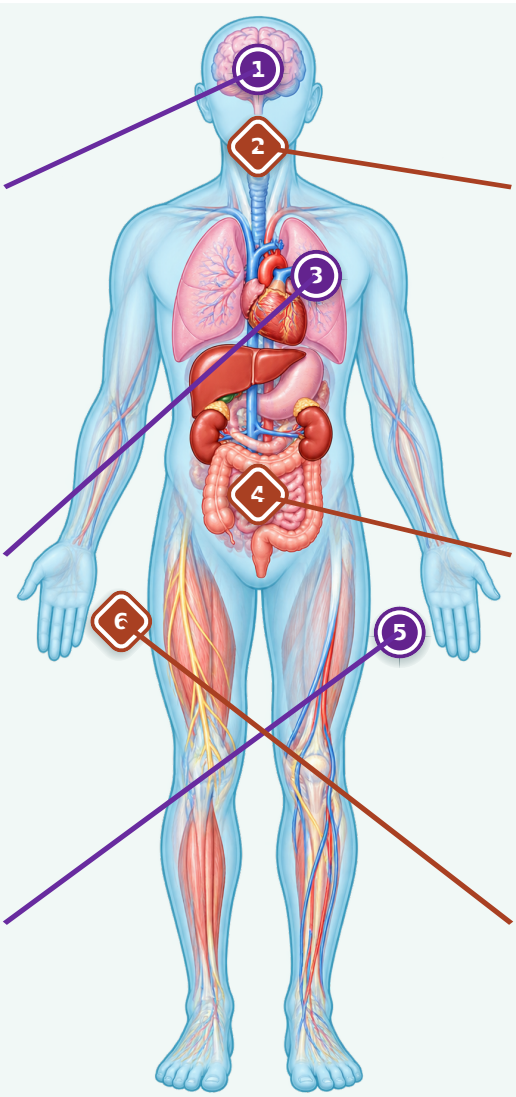
Etoposide

Tell the team about ulcers or painful swallowing; ask for help if you cannot drink.

3 Chest / breathing

Etoposide

New cough, breathlessness or chest pain needs urgent assessment.



4 Stomach / bowel

Etoposide

Report persistent diarrhoea or vomiting. Fever with severe tummy pain is urgent.

5 Skin / allergy

Etoposide

Report spreading rash; swelling or breathing difficulty needs immediate help.

6 Blood counts

Etoposide

Fever, unusual bruising or bleeding needs an immediate call. The marker is schematic.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call oncology immediately for fever or suddenly feeling unwell, unusual bleeding, severe tummy pain, persistent vomiting or diarrhoea, or new cough or breathlessness. Do not wait for the next cycle.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE NUMBERS ONLY — your team must replace these before patient use.

Keep your treatment details to hand

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Oral etoposide

Days 1–3 twice daily; 21-day cycle

Phase: _____ Cycle plan: team to confirm

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

CUT outer border

BACK / TREATMENT RECORD

My cycle record

Rolling record — follow your agreed treatment plan.

Cycle	Date / initials

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE NUMBERS ONLY — your team must replace these before patient use.

Hospital / team: _____ Backup contact route: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

NCCP 00388 · VERSION 4

Your treatment, explained

Oral etoposide · Days 1–3 twice daily

21-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology immediately for fever or suddenly feeling unwell, unusual bleeding, severe tummy pain, persistent vomiting or diarrhoea, or new cough or breathlessness. Do not wait for the next cycle.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: nccp · smpc

Choose a topic to read

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- Your treatment course
- Your 21-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- My treatment
- My schedule
- Between treatments
- When to call
- Tests and review
- Supportive medicines
- Understanding the evidence
- Questions to ask
- Sources

UNDERSTAND YOUR TREATMENT

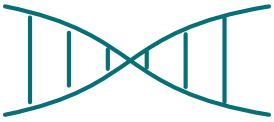
Your chemotherapy

This treatment aims to slow or shrink extensive small-cell lung cancer when intravenous or combination chemotherapy is unsuitable. Benefit varies; your team explains the aim for you.

TOPOISOMERASE-II INHIBITOR CHEMOTHERAPY

Etoposide capsules

Supplied brand may vary



Disrupts DNA copying and repair

Etoposide interferes with topoisomerase II, an enzyme involved in DNA structure and repair. DNA damage can stop cancer cells dividing. Healthy blood-forming and other cells can also be affected. Absorption by mouth varies, so oral and intravenous doses are not interchangeable.

Medicine reaches cells → DNA damage persists → Cell division disrupted

What to watch for

Call oncology immediately for fever or suddenly feeling unwell, unusual bleeding, severe tummy pain, persistent vomiting or diarrhoea, or new cough or breathlessness. Do not wait for the next cycle.

Sources: nccp · smpc

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

THE BIGGER TREATMENT PLAN

Your treatment course

REVIEW EACH CYCLE

Chemotherapy course



Take your prescribed etoposide capsules twice daily on Days 1–3. Days 4–21 have no planned etoposide. The source cycle is 21 days; continue only while helping and tolerable, after team review.

INDIVIDUAL PLAN

Review and next plan



Your team reviews benefit and side effects and agrees whether to continue, change or stop.

Relative educational plan; your team confirms actual appointments and changes.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

My tests and checks

Check	Before treatment	During the cycle	Before next course	After treatment
Safety blood tests	Blood count, kidney and liver profile	Extra tests if needed; oral etoposide regular dates confirmed by team	Blood count, kidney and liver review	Follow abnormal results as advised
Disease assessment	Confirm treatment aim	Symptoms reviewed	Scans or other tests on consultant plan	Agree next treatment or follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00388 v4 · official regimen

THE WHOLE TREATMENT JOURNEY

Oral etoposide · Days 1–3 twice daily: treatment and checks

Extensive SCLC · Days 1–3 twice daily

Take your prescribed etoposide capsules twice daily on Days 1–3. Days 4–21 have no planned etoposide. The source cycle is 21 days; continue only while helping and tolerable, after team review.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment days	Between courses	Course review
Treatment NCCP 00388	Confirm capsule strength, number, dosing days, storage and contact numbers with the pharmacist.	Take your prescribed etoposide capsules twice daily on Days 1–3. Days 4–21 have no planned etoposide. The source cycle is 21 days; continue only while helping and tolerable, after team review.	Follow support plan and report symptoms	Review benefit/toxicity; next plan
Safety tests NCCP 00388	Blood count, kidney/liver tests	Review symptoms; extra tests if needed	Blood count, kidney and liver tests before treatment and regularly at team-specified intervals. Scans and follow-up follow the consultant plan.	Follow abnormal results

What each check is for

1

Before each course

Before starting

Safety and plan may change

Ask / act: Team confirms next treatment

Sources: NCCP 00388

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-02 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.29

YOUR CYCLE, DAY BY DAY

Your 21-day cycle.

Select a day to explore what to watch for. These are possible patterns, not a promise about when symptoms start or stop.

> The full information is available below. Interactive day, phase and date controls require JavaScript; use “Read the whole cycle as text” for the complete timeline.

Your date stays on this page only. It is not saved or added to a QR code.

Medicines on Day 1: Etoposide capsules

On a small screen, swipe the chart sideways. Each day is a keyboard-accessible button.





Prescribed capsules	IV
No planned chemotherapy	Blood-count and symptom recovery varies.

Effects vary; no risk curve or guaranteed recovery day is shown.

DAY 1

Treatment days

Take your prescribed etoposide capsules twice daily on Days 1–3. Days 4–21 have no planned etoposide. The source cycle is 21 days; continue only while helping and tolerable, after team review.

Your next step: Follow the confirmed prescription; report problems promptly.

The next cycle starts 21 days after this one, if your team confirms it.

> Do not use this page to calculate or change doses.

Read the whole cycle as text

Day 1–3: Treatment days

Take your prescribed etoposide capsules twice daily on Days 1–3. Days 4–21 have no planned etoposide. The source cycle is 21 days; continue only while helping and tolerable, after team review. Follow the confirmed prescription; report problems promptly.

Day 4–21: Between treatments

No planned chemotherapy; continue prescribed support and monitoring. Do not restart until the team confirms the next cycle.

Sources: nccp

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Before treatment

Confirm capsule strength, number, dosing days, storage and contact numbers with the pharmacist.



STEP 2

Treatment visit or home course

Take etoposide capsules on an empty stomach using the exact prescribed dose and days. Do not open, chew or crush them. Capsule strengths, quantities and storage follow the supplied pharmacy label. Oral and intravenous doses are not interchangeable.



STEP 3

Bring to review

Bring your medicine list, dose record and questions; report new symptoms before starting another course.

Open practical treatment guide · Printable practical guide

Sources: NCCP 00388 v4 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Sickness and bowel symptoms

Use your prescribed prevention or rescue medicine. Report persistent vomiting, diarrhoea or inability to drink; fever with severe tummy pain needs urgent assessment.

2 Blood tests and review

Blood count, kidney and liver tests are checked before treatment and regularly on dates set by your team for the selected schedule.

3 Pregnancy, fertility and medicines

Discuss fertility and contraception before treatment. Tell the team about pregnancy, breastfeeding, all medicines, supplements and vaccines. Check before changing any medicine.

4 Missed or vomited capsules

Do not double a missed dose or take replacement capsules after vomiting without team advice. Record what happened, contact the oncology pharmacist/team, and follow the next scheduled dose in the agreed plan.

5 Safe handling

Take etoposide capsules on an empty stomach using the exact prescribed dose and days. Do not open, chew or crush them. Capsule strengths, quantities and storage follow the supplied pharmacy label. Oral and intravenous doses are not interchangeable. Keep away from children; ask the pharmacy how to return unused or damaged capsules.

Sources: nccp · smpc · ncis

TREATMENT REFERENCE · 01

My treatment

This treatment aims to slow or shrink extensive small-cell lung cancer when intravenous or combination chemotherapy is unsuitable. Benefit varies; your team explains the aim for you.

How it works

Etoposide interferes with topoisomerase II, an enzyme involved in DNA structure and repair. DNA damage can stop cancer cells dividing. Healthy blood-forming and other cells can also be affected. Absorption by mouth varies, so oral and intravenous doses are not interchangeable.

Sources: nccp · smpc

Evidence and limits

Older studies reported tumour responses with oral etoposide. The evidence is limited, and the different oral schedules cannot be assumed to have the same benefit or risks. A small uncontrolled study of a 21-day oral course does not establish a survival advantage or validate all three NCCP schedules. Its participants had recurrent cancer and substantial treatment toxicity; do not transfer its response percentage to an individual.

Sources: trial

TREATMENT REFERENCE · 02

My schedule

Take your prescribed etoposide capsules twice daily on Days 1–3. Days 4–21 have no planned etoposide. The source cycle is 21 days; continue only while helping and tolerable, after team review.

Administration

Take etoposide capsules on an empty stomach using the exact prescribed dose and days. Do not open, chew or crush them. Capsule strengths, quantities and storage follow the supplied pharmacy label. Oral and intravenous doses are not interchangeable.

Sources: nccp · smpc

TREATMENT REFERENCE · 03

Between treatments

Keep your prescription and contact plan to hand.

Sickness and bowel symptoms

Use your prescribed prevention or rescue medicine. Report persistent vomiting, diarrhoea or inability to drink; fever with severe tummy pain needs urgent assessment.

Sources: nccp · smpc

Blood tests and review

Blood count, kidney and liver tests are checked before treatment and regularly on dates set by your team for the selected schedule.

Sources: nccp · smpc

Pregnancy, fertility and medicines

Discuss fertility and contraception before treatment. Tell the team about pregnancy, breastfeeding, all medicines, supplements and vaccines. Check before changing any medicine.

Sources: nccp · smpc

Missed or vomited capsules

Do not double a missed dose or take replacement capsules after vomiting without team advice. Record what happened, contact the oncology pharmacist/team, and follow the next scheduled dose in the agreed plan.

Sources: nccp · smpc

Safe handling

Take etoposide capsules on an empty stomach using the exact prescribed dose and days. Do not open, chew or crush them. Capsule strengths, quantities and storage follow the supplied pharmacy label. Oral and intravenous doses are not interchangeable. Keep away from children; ask the pharmacy how to return unused or damaged capsules.

Sources: nccp · smpc

TREATMENT REFERENCE · 04

When to call

Call oncology immediately for fever or suddenly feeling unwell, unusual bleeding, severe tummy pain, persistent vomiting or diarrhoea, or new cough or breathlessness. Do not wait for the next cycle.

Blood counts

An infection can be serious even after treatment days have finished. A normal earlier blood test does not rule out a new problem.

Sources: nccp · smpc

TREATMENT REFERENCE · 05

Tests and review

Blood count, kidney and liver tests before treatment and regularly at team-specified intervals. Scans and follow-up follow the consultant plan.

Scans and tumour markers

Your consultant decides which are relevant and when. Do not assume a fixed imaging interval or that tumour markers are required for everyone.

Sources: nccp · smpc

After treatment

Agree follow-up, symptom support and the next plan before the course ends.

Sources: nccp · smpc

Why this treatment may be offered

Older studies reported tumour responses with oral etoposide. The evidence is limited, and the different oral schedules cannot be assumed to have the same benefit or risks.

A small uncontrolled study of a 21-day oral course does not establish a survival advantage or validate all three NCCP schedules. Its participants had recurrent cancer and substantial treatment toxicity; do not transfer its response percentage to an individual.

Sources: trial · nccp

PREPARE FOR THE CONVERSATION

Questions worth asking

Why can treatment be delayed?

Blood results or symptoms may make a delay or dose change safer. Your team confirms when to restart.

Are capsules gentler than an infusion?

No. Oral chemotherapy can cause serious toxicity and needs blood tests and review. Oral and intravenous doses cannot be exchanged.

When will I have scans?

Your consultant sets the timing for your disease and treatment plan. No universal scan or tumour-marker interval is specified here.

> **Try explaining the plan back:** Explain which days you take treatment, when your next blood tests are due and whom you will call tonight if unwell.

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00388 v4. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00388 v4 · official regimen
- HPRA Vepesid product information
- NCCP NCIS medical oncology antiemetic guidance V8
- Johnson 1990 oral etoposide phase II study

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-02.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6911/388_oral_etoposide_.pdf

https://assets.hpra.ie/products/Human/30580/Licence_PA2239-006-001_27022024163103.pdf

https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf

https://pubmed.ncbi.nlm.nih.gov/2170589/

BASELINE → TREATMENT → FOLLOW-UP

My tests and checks

Check	Before treatment	During the cycle	Before next course	After treatment
Safety blood tests	Blood count, kidney and liver profile	Extra tests if needed; oral etoposide regular dates confirmed by team	Blood count, kidney and liver review	Follow abnormal results as advised
Disease assessment	Confirm treatment aim	Symptoms reviewed	Scans or other tests on consultant plan	Agree next treatment or follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00388 v4 · official regimen

THE WHOLE TREATMENT JOURNEY

Oral etoposide · Days 1–3 twice daily: treatment and checks

Extensive SCLC · Days 1–3 twice daily

Take your prescribed etoposide capsules twice daily on Days 1–3. Days 4–21 have no planned etoposide. The source cycle is 21 days; continue only while helping and tolerable, after team review.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment days	Between courses	Course review
Treatment NCCP 00388	Confirm capsule strength, number, dosing days, storage and contact numbers with the pharmacist.	Take your prescribed etoposide capsules twice daily on Days 1–3. Days 4–21 have no planned etoposide. The source cycle is 21 days; continue only while helping and tolerable, after team review.	Follow support plan and report symptoms	Review benefit/toxicity; next plan
Safety tests NCCP 00388	Blood count, kidney/liver tests	Review symptoms; extra tests if needed	Blood count, kidney and liver tests before treatment and regularly at team-specified intervals. Scans and follow-up follow the consultant plan.	Follow abnormal results

What each check is for

1 Before each course

Before starting

Safety and plan may change

Ask / act: Team confirms next treatment

Sources: NCCP 00388

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-02 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.29

Oral etoposide · Days 1–3 twice daily

> **Check your exact treatment** This guide covers only Days 1–3 twice daily. Do not combine it with another etoposide schedule.
>
> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Separate scripts for each phase

Choose the active phase in the clinician workflow. Each script has its own medicines, timing and review checks.

- **Oral etoposide Days 1–3 twice daily support** — low etoposide

https://sactcheck.com/patient/00388/schedule_3d/supportive-care.html

Oral etoposide · Days 1–3 twice daily

NCCP 00388 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before treatment

Confirm capsule strength, number, dosing days, storage and contact numbers with the pharmacist.

STEP 2

Treatment visit or home course

Take etoposide capsules on an empty stomach using the exact prescribed dose and days. Do not open, chew or crush them. Capsule strengths, quantities and storage follow the supplied pharmacy label. Oral and intravenous doses are not interchangeable.

STEP 3

Bring to review

Bring your medicine list, dose record and questions; report new symptoms before starting another course.

Sources: NCCP 00388 v4 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

Oral etoposide · Days 1–3 twice daily

A record of what you experience

One cycle per page. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal CTCAE grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Take your prescribed etoposide capsules twice daily on Days 1–3. Days 4–21 have no planned etoposide. The source cycle is 21 days; continue only while helping and tolerable, after team review.

The four pages are reusable recording sheets, not a prescribed four-cycle course. Ask for additional sheets as needed.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital’s instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Your own hospital / contact instructions:

https://sactcheck.com/patient/00388/schedule_3d/

Full patient information, sources and visual timeline. No identifiers are encoded.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

Oral etoposide · Days 1–3 twice daily

Cycle number: _____ Start date: _____ Next review: _____

Record the actual cycle and any omitted, delayed or changed doses with your team. A completed stamp does not authorise another dose.

Etoposide capsules · Topoisomerase-II inhibitor chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999
Nausea	☐ None	☐ Queasy but eating and drinking	☐ Intake reduced: contact the team	☐ Unable to maintain intake: urgent review	☐ Collapse / severe dehydration: 112/999
Diarrhoea	☐ Usual pattern	☐ Up to 3 extra stools above normal	☐ 4–6 extra stools/day or at night: call	☐ 7+ extra stools/day or dehydration: urgent review	☐ Collapse / severe dehydration: 112/999
Mouth soreness / ulcers	☐ None	☐ Mild soreness; eating normally	☐ Painful; soft food needed: call	☐ Cannot eat / drink enough: urgent review	☐ Major breathing difficulty: 112/999
Bleeding / bruising	☐ None	☐ New small bruise / brief bleed: report	☐ Repeated bleeding: call today	☐ Bleeding that will not stop: urgent help	☐ Heavy bleeding / collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Oral etoposide · Days 1–3 twice daily

Cycle number: _____ Start date: _____ Next review: _____

Record the actual cycle and any omitted, delayed or changed doses with your team. A completed stamp does not authorise another dose.

Etoposide capsules · Topoisomerase-II inhibitor chemotherapy

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Dates / doses missed or changed / symptoms affecting daily life / questions:

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Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999
Nausea	☐ None	☐ Queasy but eating and drinking	☐ Intake reduced: contact the team	☐ Unable to maintain intake: urgent review	☐ Collapse / severe dehydration: 112/999
Diarrhoea	☐ Usual pattern	☐ Up to 3 extra stools above normal	☐ 4–6 extra stools/day or at night: call	☐ 7+ extra stools/day or dehydration: urgent review	☐ Collapse / severe dehydration: 112/999
Mouth soreness / ulcers	☐ None	☐ Mild soreness; eating normally	☐ Painful; soft food needed: call	☐ Cannot eat / drink enough: urgent review	☐ Major breathing difficulty: 112/999
Bleeding / bruising	☐ None	☐ New small bruise / brief bleed: report	☐ Repeated bleeding: call today	☐ Bleeding that will not stop: urgent help	☐ Heavy bleeding / collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

> Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Oral etoposide · Days 1–3 twice daily

Cycle number: _____ Start date: _____ Next review: _____

Record the actual cycle and any omitted, delayed or changed doses with your team. A completed stamp does not authorise another dose.

Etoposide capsules · Topoisomerase-II inhibitor chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999
Nausea	☐ None	☐ Queasy but eating and drinking	☐ Intake reduced: contact the team	☐ Unable to maintain intake: urgent review	☐ Collapse / severe dehydration: 112/999
Diarrhoea	☐ Usual pattern	☐ Up to 3 extra stools above normal	☐ 4–6 extra stools/day or at night: call	☐ 7+ extra stools/day or dehydration: urgent review	☐ Collapse / severe dehydration: 112/999
Mouth soreness / ulcers	☐ None	☐ Mild soreness; eating normally	☐ Painful; soft food needed: call	☐ Cannot eat / drink enough: urgent review	☐ Major breathing difficulty: 112/999
Bleeding / bruising	☐ None	☐ New small bruise / brief bleed: report	☐ Repeated bleeding: call today	☐ Bleeding that will not stop: urgent help	☐ Heavy bleeding / collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Keep your treatment details to hand

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Oral etoposide

Days 1–3 twice daily; 21-day cycle

Phase: _____ Cycle plan: team to confirm

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

CUT outer border

BACK / TREATMENT RECORD

My cycle record

Rolling record — follow your agreed treatment plan.

Cycle	Date / initials

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE NUMBERS ONLY — your team must replace these before patient use.

Hospital / team: _____ Backup contact route: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.31 · NCCP 00388 v4 · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.